SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Temozolomide with radiotherapy, then adjuvant temozolomide

Adults with newly diagnosed glioblastoma under NCCP 00334. · NCCP 00334 · version 4 · 28-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
This treatment combines daily temozolomide with radiotherapy, followed by a break and further temozolomide cycles, to help control newly diagnosed glioblastoma. Your team discusses the expected benefit and how it fits after surgery or biopsy.

Schedule
Take temozolomide daily throughout six weeks of radiotherapy, including weekends as prescribed. After a four-week break, take temozolomide on Days 1–5 of each 28-day cycle for up to six adjuvant cycles.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Temozolomide
ORAL CHEMOTHERAPY CAPSULE

- ☐ Low blood counts, infection and bleeding
- ☐ Nausea / vomiting and appetite loss
- ☐ Fatigue, headache and constipation
- ☐ Liver injury, including delayed injury
- ☐ PJP and other opportunistic infections
- ☐ Rash and rare severe skin reactions
- ☐ Fertility effects and harm to a pregnancy
- ☐ Rare serious bone-marrow disorders

Radiotherapy
TREATMENT FOCUSED ON THE BRAIN TUMOUR AREA

- ☐ Fatigue and scalp reaction
- ☐ Hair loss in the treated area
- ☐ Swelling and neurological symptoms
- ☐ Possible delayed effects on thinking and function

Discuss treatment intent, evidence and alternatives; phase-specific schedules; blood and organ monitoring; infection, nausea, fertility and pregnancy precautions; practical support and any unlicensed use. Keep the separate radiotherapy consent and medication plans where applicable.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00334 v4.

SACTCheck v0.84.38
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